

HOUSE No. 2196**The Commonwealth of Massachusetts**

PRESENTED BY:

Marjorie C. Decker and John J. Lawn, Jr.*To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:*

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act relative to preventing overdose deaths and increasing access to treatment.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>Marjorie C. Decker</i>	<i>25th Middlesex</i>	<i>1/17/2025</i>
<i>John J. Lawn, Jr.</i>	<i>10th Middlesex</i>	<i>9/25/2025</i>
<i>Mindy Domb</i>	<i>3rd Hampshire</i>	<i>1/23/2025</i>
<i>Lindsay N. Sabadosa</i>	<i>1st Hampshire</i>	<i>1/27/2025</i>
<i>Natalie M. Higgins</i>	<i>4th Worcester</i>	<i>1/29/2025</i>
<i>Erika Uyterhoeven</i>	<i>27th Middlesex</i>	<i>1/29/2025</i>
<i>Danillo A. Sena</i>	<i>37th Middlesex</i>	<i>1/31/2025</i>
<i>Samantha Montaño</i>	<i>15th Suffolk</i>	<i>2/3/2025</i>
<i>Christine P. Barber</i>	<i>34th Middlesex</i>	<i>2/6/2025</i>
<i>James B. Eldridge</i>	<i>Middlesex and Worcester</i>	<i>2/10/2025</i>
<i>James C. Arena-DeRosa</i>	<i>8th Middlesex</i>	<i>2/12/2025</i>
<i>Manny Cruz</i>	<i>7th Essex</i>	<i>2/26/2025</i>
<i>Antonio F. D. Cabral</i>	<i>13th Bristol</i>	<i>2/27/2025</i>
<i>Michael D. Brady</i>	<i>Second Plymouth and Norfolk</i>	<i>2/27/2025</i>
<i>Paul McMurtry</i>	<i>11th Norfolk</i>	<i>3/3/2025</i>
<i>Steven Owens</i>	<i>29th Middlesex</i>	<i>3/3/2025</i>
<i>Natalie M. Blais</i>	<i>1st Franklin</i>	<i>3/5/2025</i>
<i>Adrian C. Madaro</i>	<i>1st Suffolk</i>	<i>3/11/2025</i>

<i>Steven Ultrino</i>	<i>33rd Middlesex</i>	<i>3/11/2025</i>
<i>Richard M. Haggerty</i>	<i>30th Middlesex</i>	<i>3/11/2025</i>
<i>Sean Reid</i>	<i>11th Essex</i>	<i>3/19/2025</i>
<i>Priscila S. Sousa</i>	<i>6th Middlesex</i>	<i>3/19/2025</i>
<i>Adam J. Scanlon</i>	<i>14th Bristol</i>	<i>3/26/2025</i>
<i>Christopher Hendricks</i>	<i>11th Bristol</i>	<i>4/7/2025</i>
<i>Margaret R. Scarsdale</i>	<i>1st Middlesex</i>	<i>4/7/2025</i>
<i>Russell E. Holmes</i>	<i>6th Suffolk</i>	<i>4/7/2025</i>
<i>James J. O'Day</i>	<i>14th Worcester</i>	<i>4/7/2025</i>
<i>John F. Keenan</i>	<i>Norfolk and Plymouth</i>	<i>6/11/2025</i>
<i>Chynah Tyler</i>	<i>7th Suffolk</i>	<i>4/28/2025</i>
<i>Kate Donaghue</i>	<i>19th Worcester</i>	<i>5/8/2025</i>
<i>Tara T. Hong</i>	<i>18th Middlesex</i>	<i>5/12/2025</i>
<i>Estela A. Reyes</i>	<i>4th Essex</i>	<i>5/28/2025</i>
<i>Jack Patrick Lewis</i>	<i>7th Middlesex</i>	<i>6/11/2025</i>
<i>Greg Schwartz</i>	<i>12th Middlesex</i>	<i>9/3/2025</i>
<i>John J. Marsi</i>	<i>6th Worcester</i>	<i>9/12/2025</i>
<i>Amy Mah Sangiolo</i>	<i>11th Middlesex</i>	<i>9/16/2025</i>
<i>Carmine Lawrence Gentile</i>	<i>13th Middlesex</i>	<i>9/16/2025</i>
<i>Rodney M. Elliott</i>	<i>16th Middlesex</i>	<i>1/9/2026</i>
<i>Michelle L. Badger</i>	<i>1st Plymouth</i>	<i>3/16/2026</i>

HOUSE No. 2196

By Representatives Decker of Cambridge and Lawn of Watertown, a petition (accompanied by bill, House, No. 2196) of Marjorie C. Decker, Mindy Domb and others relative to overdose deaths and access to treatment. Mental Health, Substance Use and Recovery.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to preventing overdose deaths and increasing access to treatment.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 111 of the General Laws, as appearing in the 2022 edition, is
2 hereby amended by inserting after section 215 the following new section:-

3 Section 215A. (a) For the purposes of this section, the following words shall have the
4 following meanings unless the context clearly requires otherwise:

5 “Harm reduction program”, a program or service that reduces the adverse consequences
6 of substance use, including use-related morbidity and mortality, stabilizes and improves the
7 health of people who use substances, and advances public health. Programming or services
8 provided by a harm reduction program may include, but shall not be limited to: needle exchange;
9 primary care, including disease prevention and health screenings; access or referral to evidence-
10 based treatment options; drug testing services; overdose reversal care; supervision of persons
11 who use pre-obtained substances; and other social support services deemed permissible by the
12 department.

“Harm reduction program operators”, entities or individuals directly involved in the operation, administration or staffing of a harm reduction program, including directors, board members, consultants, health care providers, service providers, staff and volunteers.

(b) Not later than 1 year after the implementation of a harm reduction program under this subsection, the department shall report the results of authorized programs pursuant to this section and any recommendations by filing the report with the clerks of the senate and house of representatives, the joint committee on mental health, substance use and recovery, the joint committee on public health and the senate and house committees on ways and means.

(c) Notwithstanding any general or special law to the contrary, harm reduction program operators, individuals who access harm reduction program services, owners, lessors and sub-lessors of property used for harm reduction programs and state, county and municipal employees involved in approving or operating harm reduction programs shall, for actions related to the approval or operation of, or participation in, a harm reduction program, be immune from: (i) arrest, charge or prosecution, including for attempting, aiding and abetting or conspiracy to commit a violation, pursuant to sections 32, 32A, 32B, 32C, 32D, 32E, 32I, 34, 40, 43 and 47 of chapter 94C and chapter 271A; (ii) seizure or forfeiture of data, records, assets or property under state law; (iii) civil suit, liability or damages alleged to have been sustained by an act or omission by a harm reduction program operator in the course of providing harm reduction services; (iv) denial of any right or privilege; and (v) for health care providers, disciplinary action by a professional licensing board, credentialing restriction, contractual liability, adverse employment action or denial of any professional privilege; provided, however, that the immunity described in this subsection shall apply only if the harm reduction program is approved by the department and

operates in good faith in accordance with this section and regulatory requirements issued by the department.

(d) Entering or exiting or utilizing a harm reduction program cannot serve as the basis for, or a fact contributing to the existence of, reasonable suspicion or probable cause to conduct a search or seizure.

(e) The immunity provided under subsection (c) shall not apply: (i) if the damage was caused by an act or omission constituting gross negligence or recklessness, conduct with an intent to harm, discrimination based on race, ethnicity, national origin, religion, disability, sexual orientation or gender identity, or conduct outside the scope of responsibility of a harm reduction program employee or volunteer, as determined by the department; (ii) to consumer protection actions brought by the attorney general; (iii) to false claims actions brought by or on behalf of the commonwealth; or (iv) privacy violations.

(f) Notwithstanding any general law or special law to the contrary, a person or entity providing harm reduction services under this section and approved by the department shall not be required to register their activities pursuant to section 7 of chapter 94C.

(g) The department shall promulgate regulations to implement this section.

SECTION 2. Section 193U of chapter 175 of the general laws, as amended by section 23 of chapter 127 of the acts of 2022, is hereby amended by inserting in the first sentence of the second paragraph, after the word “that,”, the following words:- “a health care provider provides services at a harm reduction program, or”.